Vet.App. No. 19-4044

IN THE UNITED STATES COURT OF APPEALS FOR VETERANS CLAIMS

CRAIG S. RUNDELL,
Appellant,

v.

ROBERT L. WILKIE,
Secretary of Veterans Affairs,
Appellee.

ON APPEAL FROM THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

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v.	)	Vet. App. No. 19-4044
ROBERT L. WILKIE, Secretary of Veterans Affairs, Appellee.	)	

ON APPEAL FROM
THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

I. ISSUES PRESENTED

Whether the Court should affirm the February 26, 2019, Board of Veterans' Appeals (Board) decision that denied service connection for (1) an eating disorder, claimed as weight gain, (2) sleep apnea, (3) diverticulitis and (4) a headache disorder.

II. STATEMENT OF THE CASE

A. Jurisdiction

The Court has jurisdiction over this matter pursuant to 38 U.S.C. § 7252(a).

B. Nature of the Case

Craig S. Rundell (Appellant) appeals the February 26, 2019, Board decision to the extent that it denied service connection for an eating disorder, claimed as weight gain, sleep apnea, diverticulitis and a headache disorder. (*See* Record

((R.) at 1-16 (Board decision)).[1] Appellant contends only that the VA examinations and opinions, upon which the Board's decision was largely premised, were inadequate. (Appellant's Brief (App. Br.) at 2-9). The Secretary disputes this contention.

C. Statement of Relevant Facts

Appellant served on active duty from March to April 2008. (R. at 1783). In October 2016, Appellant filed a claim, seeking service connection for sleep apnea, migraine headaches, diverticulitis, and weight gain, all as secondary to his service-connected post-traumatic stress disorder (PTSD). (R. at 851 (847-51)).

Appellant underwent VA examination as to each of these issues in December 2016 and the examiner proffered negative nexus opinions as to whether any of the conditions were caused or aggravated by his PTSD. (R. at 576-89, 594-96). A rating decision the same month denied service connection. (R. at 554 (552-58)). Appellant filed a notice of disagreement (NOD) in February 2017. (R. at 502-511). VA issued a statement of the case (SOC) the next month and Appellant perfected his appeal the month after. (R. at 275 (274-76), 280-317).

In the February 2019 decision now before the Court, the Board denied service connection as to the disabilities on appeal. (R. at 5-14). Regarding Appellant's claim for weight gain, the Board found that Appellant had not been

[1] The Board's decision to reopen Appellant's claim for service connection for an eating disorder is a favorable finding that cannot be disturbed by the Court. *Medrano v. Nicholson*, 21 Vet.App. 165, 170-71 (2007).

diagnosed with an eating disorder despite examination therefor, and that weight gain was not a disability for which service connection could be granted. (R. at 5, 10-13). Regarding Appellant's other claims, the Board found that the preponderance of the evidence of record was against a finding that they were either caused or aggravated by Appellant's PTSD. (R. at 5, 11-13). This appeal followed.

III. SUMMARY OF THE ARGUMENT

The Court should affirm the Board's decision because the VA examination reports and opinions, upon which the Board relied, were adequate to adjudicate the issues on appeal as they were based on Appellant's medical history and supported by adequate rationales. Appellant generally argues with the conclusions the examiners reached and assumes a reasons or bases requirement where none exists. He has not demonstrated that the examination reports or the corresponding medical opinions were inadequate under the relevant legal principles. Thus, Appellant has failed to meet his burden of demonstrating error in the Board's decision.[2]

[2] Appellant does not allege that any of the Board's findings are clearly erroneous or that the Board's statement of reasons or bases is preclusive of judicial review. He thus waives these arguments. *See Disabled Am. Veterans v. Gober*, 234 F.3d 682, 688 n. 3 (Fed. Cir. 2000) (stating that the Court would "only address those challenges that were briefed").

IV. ARGUMENT

A. The VA Examinations and Opinions of Record Were Adequate To Allow the Board To Adjudicate the Issues on Appeal

A medical examination or opinion is adequate where it is based upon consideration of a claimant's medical history, describes the disability, if any, in sufficient detail so that the Board's evaluation will be fully informed, and informs the Board of the examiner's judgment on a medical question and the essential rationale for that opinion. *Stefl v. Nicholson*, 21 Vet.App. 120, 123 (2007); *Monzingo v. Shinseki*, 26 Vet.App. 97, 105 (2012) (per curiam). Although VA examiners must provide more than bare assertions, "there is no reasons or bases requirement imposed on examiners." *Acevedo v. Shinseki*, 25 Vet.App. 286, 293 (2012). An examination report must be read as a whole and there is no obligation on the examiner to explicitly lay out their journey from the facts to the conclusions. *Monzingo*, 26 Vet.App. at 106.

"Whether a medical [examination or] opinion is adequate is a finding of fact, which the Court reviews under the 'clearly erroneous' standard." *D'Aries v. Peake*, 22 Vet.App. 97, 104 (2008) (per curiam); *see* 38 U.S.C. § 7261(a)(4)). Under such standard, if there is a 'plausible' basis in the record for the Board's factual determinations, even if the Court might not have reached the same conclusions, it cannot overturn them. *Gilbert v. Derwinski*, 1 Vet.App. 49, 52-53 (1990); *see Forcier v. Nicholson*, 19 Vet.App. 414, 421 (2006). Therefore, the Court can overturn the Board's decision "only when there is no 'plausible basis in the record'

for the decision." *Smallwood v. Brown*, 10 Vet.App. 93, 97 (1997) (quoting *Gilbert*, 1 Vet.App. at 53); *see also Anderson v. City of Bessemer City*, *N.C.,* 470 U.S. 564, 574 (1985) (holding that "[w]here there are two permissible views of the evidence, the factfinder's choice between them cannot be clearly erroneous").

Ultimately, the burden is on Appellant to demonstrate error in the Board's reliance on the examinations and opinions of record. *See Martinak v. Nicholson*, 21 Vet.App. 447, 456 (2007) ("The appellant bears the burden of demonstrating on appeal any prejudice caused by a deficiency in an examination"). Here, the examinations and opinion upon which the Board relied were adequate to decide the issues on appeal. As the Board noted in its decision, the 2016 opinions were predicated on a thorough review of the record, clearly reflected consideration of Appellant's medical records, and provided clear conclusions with rationales that were supported by the evidence of record and reasoned medical explanations. (R. at 12). The Board therefore placed great probative values on these opinions. (*Id.*); *Stefl,* 21 Vet.App. at 123-24; *see Nieves-Rodriguez v. Peake*, 22 Vet.App. 295, 300-01 (2008).

1. Eating disorder claimed as weight gain

In order to establish service connection on any basis there must first be a current disability. 38 C.F.R. §§ 3.303, 3.310. "In the absence of proof of a current disability, there is no valid claim." *Brammer v. Derwinski*, 3 Vet.App. 223, 225 (1992). Appellant has claimed service connection for weight gain, (R. at 847-51) and has argued that his weight fluctuation should be service connected. (App. Br.

at 3-5). Weight gain is not a disability in and of itself for which service connection may be granted. VA Office of General Counsel Precedential Opinion (VAOPGCPREC) 1-2017 ("the fact that VA has not included obesity or weight gain as ratable conditions in its schedule for rating disabilities indicates that VA does not view obesity or weight gain itself as constituting a 'disability'"); *see Marcelino v. Shulkin*, 29 Vet.App. 155, 158 (2018) ("As there is currently no provision in the rating schedule to compensate for obesity, a ruling by this Court finding that VA should include obesity as a compensable disease or a finding that the Board erred when it denied Mr. Marcelino service connection for obesity would require the Court to undertake the very review of the rating schedule that has been barred from its jurisdiction.").

Thus, in construing Appellant's claim in the broadest possible sense, VA ordered a medical examination to determine whether Appellant suffered from a possible psychiatric eating disorder. For such a psychiatric condition to receive service connection however, it must first be diagnosed by a medical professional in accordance with the principles of the Diagnostic and Statistical Manual for Mental Disorders, Fifth Edition (DSM-5). *See* 38 C.F.R. § 4.125. In this case, the December 2016 examiner, a Board-certified Psychologist, concluded, based on a review of Appellant's file, an in-person interview, and his own medical judgment, that Appellant did not meet the criteria for a diagnosis of an eating disorder under the DSM-5. (R. at 595-96). The VA examination report was fully adequate to decide the issue, as found by the Board. (R. at 12).

The psychologist interviewed and assessed Appellant and reviewed Appellant's file and medical records. (R. at 594). The psychologist noted and discussed relevant evidence of record, including Appellant's lay statements regarding his weight gain and loss, his eating habits, his level of physical activity and other circumstances surrounding his weight gain and loss. (*Id*.). The examiner also noted that Appellant had never been diagnosed with an eating disorder. (R. at 595-96). This is despite having been evaluated and treated by multiple mental healthcare professionals for other psychiatric conditions between 2010 and 2016. (*see e.g.* R. at 623-25, 652-54, 1306-09). The examiner noted other more likely causes for Appellant's weight issues, including very poor eating habits and a reduction in physical activity since separation from service, as well as other factors that would impact weight gain or loss, including genetics and metabolism. (R. at 595). Most importantly, the psychologist found that "[t]he results of today's evaluation indicated the Vet does not currently meet diagnostic criteria for any DSM-5 eating disorder, including binge eating disorder." (*Id*.). The examiner's conclusion was based on an application of his medical expertise to the evidence of record and supported by thorough and detailed opinion that allowed for a fully informed consideration by the Board. *See Stefl*, 21 Vet.App. at 123. Given that the examiner was adequately apprised of Appellant's medical history, had directly interviewed Appellant, and explained the reasoning for his conclusions, it is difficult to imagine what more the physician could or should have said regarding Appellant's diagnosis or lack thereof. *Monzingo* 26 Vet.App. at 105

(holding that an adequate medical examination "sufficiently inform[s] the Board of a medical expert's judgment on a medical question and the essential rationale for that opinion.").

Appellant objects to the adequacy of the opinion, but ultimately fails to prove that it is inadequate under applicable legal provisions. Appellant maintains that the examiner "does not consider [Appellant's] full medical history." (App. Br. at 5). However, the examiner reviewed Appellant's file and listened to Appellant report his medical history at the examination, which the examiner recorded, obviously considered, and indeed specifically addressed in his rationale. (R. at 594-95). Appellant also faults the examiner for providing a rationale "without reporting, addressing, or otherwise analyze [Appellant's] history of weight gain and loss, and makes no comment about this history." (App. Br. at 4). First, the examiner did discuss Appellant's medical history, including, *inter alia*, his weight gain, weight loss, lack of exercise. *Acevedo*, 25 Vet.App. at 294 (noting that medical examination reports "must be read as a whole" in determinations of adequacy). Moreover, the examiner is not required to "report address or analyze" the evidence of record in making a diagnosis. "[T]here is no reasons or bases requirement imposed on examiners. Rather, an adequate medical report must rest on correct facts and reasoned medical judgment." *Acevedo*, 25 Vet.App. at 293. Here, Appellant fails to demonstrate any misstatement of fact by the psychologist or any competent evidence that would call into question the examiners medical judgment. *See Rizzo v. Shinseki*, 580 F.3d 1288, 1291 (Fed. Cir. 2009) (holding that a VA

medical professional is presumed competent in the absence of clear evidence to the contrary). Moreover, a medical examiner is not required to comment on every piece of favorable evidence or lay out his journey from facts to a conclusion in order to render a valid medical opinion. *Roberson v. Shinseki*, 22 Vet.App. 358, 366 (2009); *see Monzingo*, 26 Vet.App. at 105-06. Thus, notwithstanding Appellant's argument to the contrary, the examiner's opinion was not inadequate simply because he did not mention each specific record cited by Appellant.

Appellant clearly disagrees with the examiner's opinion, however simply disagreeing with an examiner's opinion does not render the opinion inadequate. *Monzingo*, 26 Vet.App. at 106. While Appellant cites to a history of weight fluctuation, he proffers no competent evidence that a diagnosis of an eating disorder is warranted or has ever been made despite, as noted above, the fact that Appellant has been seen repeatedly by multiple mental health practitioners for treatment and/or diagnostic evaluation, and none of them have diagnosed him with an eating disorder. Moreover, neither Appellant nor his counsel have demonstrated that they are qualified in any way to render a competent opinion as to whether he meets the criteria for an eating disorder under the DSM-5. *Kern v. Brown*, 4 Vet.App. 350, 353 (1993) (noting that neither counsel nor the appellant were qualified to explain the significance of medical evidence).

Finally, Appellant argues that the examiner failed to provide "supporting rationale or data to support her opinion that [Appellant's] weight gain and eating disorder are unrelated to his MDD/PTSD." (App. Br. at 5). The examiner did in

fact provide such rationale, as discussed above, regarding which Appellant has identified no factual inaccuracy, or competent contrary opinion. Moreover, any deficit within the rationale provided as to a relationship between Appellant's weight gain/loss to MDD/PTSD is a harmless error, as without a valid current diagnosis there can be no service connection, even on a secondary basis. *Brammer*, 3 Vet.App. at 225. Thus, even if some direct relationship existed between Appellant's weight gain or loss and his service-connected psychiatric condition, which the examiner found did not, it is not material to the claim at hand in light of the fact that Appellant has not been diagnosed with a condition for which service connection may be granted. 38 C.F.R. §§ 3.310, 4.125.

The December 2016 examiner's report and opinon were adequate to enable the Board to make a fully informed evaluation of Appellant's claim and the Board did not clearly err by relying on them in support of its decision. *Nieves-Rodriguez*, 22 Vet.App. at 301; *Stefl*, 21 Vet.App. at 124.

2. Headaches

Appellant has claimed service connection for migraines secondary to his service-connected psychiatric disability. (R. at 851). In denying service connection for such condition, the Board again relied on the December 2016 VA examination report. (R. at 11-13). Again, the Board plausibly found the VA examination was adequate to decide the issue. (R. at 12).

Here, as noted by the Board, the examiner found that Appellant did not have migraines but had reported headaches and assessed the etiology of the reported

headaches. (R. at 11, 577, 580-582). The examiner noted that Appellant had for a time been prescribed Prazosin for nightmares associated with PTSD but that the medication had been discontinued due to headaches occurring as a side effect, and that treatment records showed Appellant repeatedly denied experiencing headaches after that. (R. at 580). The examiner also noted treatment records noting headaches but associating them with colds and sinus conditions. (R. at 581). He also discussed Appellant's current headache complaints. (*Id.*). The examiner opined that the current headaches were independent of his PTSD or any treatment therefor. (R. at 577). He again noted that Appellant had at one point reported acute headaches associated with a medication used for treatment of PTSD symptoms but that that medication had been discontinued. (*Id.*).

The examination report and opinion, when read as a whole, provides the examiner's opinion that Appellant has not been diagnosed with migraines but has had different types of headaches at different times due to acute factors such as colds, sinuses, or side effects from medication. The examiner noted that one of these acute factors was the prescription of a drug being used to treat PTSD symptoms. However, the examiner also noted that Appellant had denied ongoing headaches after the medication was discontinued. Thus, the examiner's conclusion that Appellant's current headaches are not caused or aggravated by his PTSD/MDD is supported by an adequate rationale. *Stefl*, 21 Vet.App. at 123, *Monzingo*, 26 Vet.App. at 105.

Appellant again disagrees with the examiner's conclusion, *see* (App. Br. at 6 (arguing that "[t]his conclusion is unsupportable"), but again fails to read the examination report as a whole, does not demonstrate that the opinion is based on incorrect facts, and provides no medical evidence or opinion to contradict the examiner's findings. (App. Br. at 6-7). Appellant faults the examiner for not discussing Appellant's current symptoms but cites to the examination report in his brief as evidence of those symptoms. (App. Br. at 6); *Acevedo*, 25 Vet.App. at 294. Appellant also argues that "the examiner provides no rationale for his opinion that [Appellant's] headaches are not caused by or related to [Appellant's] PTSD." (App. Br. at 6). This is obviously incorrect. The examiner explained that the only headaches which were related to Appellant's PTSD or treatment therefore were the result of medication and had resolved once the medication was discontinued, and that any current headaches were independent of Appellant's PTSD/MDD. (R. at 577, 580-82). Appellant discontinued the use of the medication causing the headaches, Prazosin, was discontinued prior to the filing of his claim. (R. at 324-25). Furthermore, to the extent Appellant argues that the examination report "violates *Stefl* because the examiner does not consider [Appellant's] full medical history" the examiner clearly not only considered, but also discussed, Appellant's history of headaches, including the timing and the acute causative factors associated thereto in the examination report and opinion. (R. at 577, 580-582).

Ultimately, while Appellant feels that the examiner's conclusion is "unsupportable," in the absence of competent evidence to the contrary, such

speculation is irrelevant. *Hyder v. Derwinski*, 1 Vet.App. 221, 225 (1991) ("Lay hypothesizing, particularly in the absence of any supporting medical authority, serves no constructive purpose and cannot be considered by this Court."). Appellant has not established that the examination report and opinion are inadequate to adjudicate the issue and has therefore not met his burden of establishing error in the Board's decision. *See Hilkert v. West*, 12 Vet.App. 145, 151 (1999) (an appellant bears the burden of persuasion to show that the Board's reliance on an examiner's opinion was in error).

3. Sleep Apnea

In denying service connection for sleep apnea as secondary to Appellant's psychiatric condition, the Board again relied on a December 2016 VA examination report and opinion, which it found to be adequate to decide the claim. (R. at 11-13, 576-79, 586-89).

Here, as noted by the Board, the December 2016 examiner adequately explained that obstructive sleep apnea was due to or the result of upper airway resistance that resulted in decreased blood oxygenation levels during sleep, causing arousals and other symptoms associated with obstructive sleep apnea. (R. at 11, 577-78). The examiner then specifically opined that there was no evidence to support that the Appellant's psychiatric disability or any treatment thereof has resulted in any upper airway resistance that would ultimately result in obstructive sleep apnea. (*Id.*). Thus, the examiner opined that it was less likely than not that the obstructive sleep apnea was due to, the result of, or aggravated

by Appellant's service-connected psychiatric disability or treatment thereof. (*Id.*). This is a sufficient rationale as the examiner explained his medical reasoning and the principles that support his conclusion, allowing the Board to weigh the opinion against contrary opinions, had there been any, and the other evidence of record. *Nieves-Rodriguez*, 22 Vet.App. at 301; *Acevedo*, 25 Vet.App. at 293.

Again, Appellant contends that there is no rationale and is again demonstrably incorrect. (App. Br. at 8). He also maintains that the examiner did not consider that his sleep symptoms began near separation, again citing to the same examination report and indicating that the report and opinion were once again not read as a whole. *Monzingo*, 26 Vet.App. at 106; (App. Br. at 8). His argument is, therefore, without any factual support.

Appellant vaguely states that the examiner did not discuss "medical journal evidence" or discuss weight fluctuation on sleep apnea. (App. Br. at 8). However, Appellant cites to no "medical journal evidence" in his brief. If he refers to a single-page internet article abstract submitted to VA after the examination report took place, the Board addressed this evidence in its decision and found it of little probative value. (R. at. 13, 501); *Acevedo*, 25 Vet.App. at 293 (holding that the Board, not the medical expert, is tasked with weighing and reconciling competing medical evidence). To the extent he refers to any other "medical journal evidence," there is no legal requirement that a medical examiner specifically identify the medical literature he or she relies upon to support his or her opinion. *Monzingo*, 26 Vet.App. at 106. Finally, as Appellant's weight gain is not a service-connected

disability, and has not been found to be related to a service-connected disability, there is no reason for the examiner to discuss the matter at all. Again, Appellant's argument with the adequacy of the VA examination report and opinion regarding sleep apnea is nothing more than a disagreement with the examiner's conclusion. As above, the examination report and opinion were based on Appellant's reported history as well as a review of the claims file, was supported by a reasoned medical explanation, and has not been shown to be based on incorrect facts. Thus, Appellant has not established that it is inadequate or that the Board erred in relying on it. *Stefl*, 21 Vet.App. at 123, *Monzingo*, 26 Vet.App. at 105.

4. Diverticulitis

Regarding the Board's denial of service connection for diverticulitis, Appellant merely asserts that the "examination is similarly conclusory," without providing any further explanation or argument. (App. Br. at 12). It is Appellant's responsibility to articulate the basis of his or her arguments and sufficiently develop those arguments to permit an informed consideration of the same. *See Locklear v. Nicholson*, 20 Vet.App. 410, 416 (2006) (holding that the Court will not entertain underdeveloped arguments). In other words, not only does he fail to meet the burden of establishing error in the Board's decision or with regard to the examination report of record, Appellant does not meet his burden of even articulating a cogent argument. As any discernible argument here is underdeveloped at best, and there is no specific allegation of error, other than a general allegation of inadequacy, and as the Secretary cannot respond where no

specific error has been suggested by counsel, the Court should decline to entertain the matter. *Coker v. Nicholson*, 19 Vet.App. 439, 442 (2006) ("The Court requires that an appellant plead with some particularity the allegation of error so that the Court is able to review and assess the validity of the appellant's arguments"). In this case, the Board denied service connection based on the examination report containing a negative nexus opinion, and Appellant fails to articulate a basis of error. (R. at 11-13); *Hilkert*, 12 Vet.App. at 151 Therefore, the Court should reject his arguments.

Therefore because the Board properly considered the evidence of record and had plausible bases for its determinations and because Appellant has not demonstrated that the Board's findings of fact are clearly erroneous or that it erroneously relied on inadequate medical reports or opinions, the Court should affirm the Board's decision.

V. CONCLUSION

For the foregoing reasons, the Court should affirm the Board's decision.

Respectfully submitted,

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